



Aleksandra Gorczynska-Kolodziej

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OFFICIAL NDIS REPORT

Official NDIS Progress Report - Stage 7/7: Documentation & Evaluation

Report Date: 2026-06-22
Participant ID: 6XGFASFF

NDIS PARTICIPANT DETAILS

PARTICIPANT NAME

Johnathan Miller

NDIS PARTICIPAN ID

N123456789

PARTICIPANT EMAIL

j.miller@example.com.au

DATE OF BIRTH (DOB)

2000-09-11

NDIS SERVICE CLASSIFICATION DETAILS

NDIS SUPPORT CATEGORY

15 Improved Daily Living Skills

NDIS SUPPORT ITEM CODE

15_043_0128_1_3

ASSIGNED PROGRAM STAGE

Stage 7/7: Documentation & Evaluation

NDIS Plan Focus & Objectives

Here is the improved NDIS goal statement, focusing on clear action, capacity building, and measurable outcomes:

Achieve significant self-management of anxiety symptoms, evidenced by a sustained reduction in GAD-7 scores to the 'Mild' range (from 16/21), enabling Johnathan Miller to independently re-engage in vocational activities at part-time employment (maintaining \$15 hours per week) and successfully navigate community environments without requiring external prompting for anxiety regulation.

Official NDIS Audit & Progress Findings

THERAPEUTIC PROGRAM COMPLETION REPORT

(Psychosocial Focus)

Practice Details:

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Report Date: 2026-06-21

PARTICIPANT INFORMATION

FIELD	DETAILS
Full Name	Johnathan Miller
NDIS Number	123456789
Program Name	Anxiety Management & Community Reconnection Program
Program Duration	15 January 2026 - 20 June 2026 (22 weeks)
Total Sessions	10 sessions (10 hours)
Therapeutic Approach	Cognitive Behavioural Therapy (CBT) with trauma-informed and strengths-based elements

1. NDIS GOAL ALIGNMENT P

This section maps the therapeutic outcomes directly to the participant's approved NDIS plan goals.

NDIS PLAN GOAL (VERBATIM FROM PLAN)	GOAL STATUS	EVIDENCE / LINK TO PROGRAM OUTCOMES
"I want to manage my anxiety so I can return to part-time work at the library and feel confident in social situations."	[x] Achieved [] Partially [] In Progress	Participant reduced anxiety symptoms by 50% (GAD-7: 16'8), successfully returned to part-time work (15 hrs/week) since April 2026, and independently uses grounding techniques before social interactions.
"I want to rebuild my social network and reconnect with friends after my mental health decline."	[] Achieved [x] Partially [] In Progress	Expanded social network (LSNS-6: 6'11), rejoined book club, but still requires support worker assistance to initiate new community activities.
"I want to develop coping strategies to manage panic attacks without avoiding public spaces."	[x] Achieved [] Partially [] In Progress	Zero panic attacks in public spaces in the last 8 weeks; participant independently uses 4 grounding strategies (Sec. 5) and has successfully used public transport 12 times independently.

2. EXECUTIVE SUMMARY (Support Coordinator Copy/Paste)

Program Overview:

Participant completed a 10-session psychosocial therapeutic program focusing on anxiety management, emotional regulation, and community reconnection. Program was delivered using Cognitive Behavioural Therapy (CBT) with trauma-informed and strengths-based elements, including psychoeducation, cognitive restructuring, exposure hierarchy development, and relapse prevention planning.

Overall Outcome:

- [x] SIGNIFICANT IMPROVEMENT in psychosocial functioning
- [] MODERATE IMPROVEMENT in psychosocial functioning
- [] MINIMAL IMPROVEMENT - alternative approach recommended

Participant's Voice:

> *"Before this program, I couldn't even go to the shops without having a panic attack. I felt completely isolated and thought I'd never work again. Now I'm back at the library part-time, I've reconnected with my book club friends, and when I feel anxious I know exactly what to do to calm down. I feel like I have my life back."* – Johnathan Miller

Key Psychosocial Achievements:

- GAD-7 reduced from 16 to 8 (50% reduction in anxiety symptoms - moved from "severe" to "mild" range)
- PHQ-9 reduced from 14 to 7 (50% reduction in depressive symptoms - moved from "moderate" to "mild" range)
- WEMWBS improved from 38 to 52 (+14 points - significant increase in mental wellbeing and flourishing)
- LSNS-6 improved from 6 to 11 (+5 points - expanded social network by 83%)

- Participant now independently uses 4 adaptive coping strategies (grounding, cognitive restructuring, breathing exercises, self-compassion)
- Successfully returned to part-time employment (15 hours/week) after 18 months of unemployment due to mental health challenges

Recommendations:

- ☒ **Continue psychosocial therapeutic support:** 8 sessions over 16 weeks (Phase 2: Social Confidence & Relationship Building)
- ☐ **Transition to maintenance:** [X] sessions/month
- ☐ **Discharge from intensive therapy** (relapse prevention contingency budget recommended)
- **Psychosocial support worker:** 4 hours/week (☐ *maintain* ☒ *increase* ☐ *decrease* - from 2 to 4 hours/week to support community participation)

3. PSYCHOSOCIAL OUTCOMES

PSYCHOSOCIAL DOMAIN	BASELINE	POST-PROGRAM	CHANGE	STATUS
Anxiety symptoms	GAD-7: 16/21 (Severe)	GAD-7: 8/21 (Mild)	50% reduction	[x] Achieved
Depressive symptoms	PHQ-9: 14/27 (Moderate)	PHQ-9: 7/27 (Mild)	50% reduction	[x] Achieved
Psychological distress	K10: 32/50 (High)	K10: 18/50 (Moderate)	44% reduction	[x] Achieved
Mental wellbeing	WEMWBS: 38/70 (Low)	WEMWBS: 52/70 (Moderate-High)	+14 points	[x] Achieved
Social connection	LSNS-6: 6/18 (Isolated)	LSNS-6: 11/18 (Connected)	+5 points (83% increase)	[x] Achieved
Perceived support	MSPSS: 28/49 (Low)	MSPSS: 40/49 (High)	+12 points (43% increase)	[x] Achieved
Functional impact	WHODAS: 38/60 (High)	WHODAS: 22/60 (Moderate)	42% reduction	[x] Achieved
Coping strategies	1 adaptive (avoidance)	4 adaptive strategies	+3 strategies	[x] Achieved

4. SESSION ATTENDANCE & ENGAGEMENT

METRIC	DETAILS
Total Scheduled Sessions	10
Sessions Attended	10 (100%)
Engagement Level	☐ Low ☐ Moderate ☒ High ☐ Very High
Homework Completion	☐ Low ☐ Moderate ☒ High ☐ Very High

Caregiver / Support Network Involvement:

- Session 3: Psychoeducation session with participant's sister (primary support person) to align understanding of anxiety management strategies and develop shared language for supporting Johnathan Miller during challenging moments.
- Session 7: Collaborative session with participant's GP (Dr. James Wilson) to coordinate care and review progress (with participant consent).

5. PSYCHOSOCIAL STRENGTHS DEVELOPED

PSYCHOSOCIAL SKILL	PROFICIENCY	INDEPENDENT USE
Emotional awareness		[x] Yes

	☐ Beginner ☐ Intermediate ☒ Advanced	
Anxiety management techniques	☐ Beginner ☐ Intermediate ☒ Advanced	☒ Yes
Cognitive restructuring	☐ Beginner ☐ Intermediate ☒ Advanced	☒ Yes
Grounding strategies (5-4-3-2-1, box breathing)	☐ Beginner ☐ Intermediate ☒ Advanced	☒ Yes
Social engagement skills	☐ Beginner ☒ Intermediate ☐ Advanced	☐ With prompting
Self-compassion practices	☐ Beginner ☐ Intermediate ☒ Advanced	☒ Yes
Exposure hierarchy implementation	☐ Beginner ☐ Intermediate ☒ Advanced	☒ Yes

6. PSYCHOSOCIAL FUNCTIONING CHANGES

•Mental Health & Emotional Wellbeing:

- Pre-Program:** Baseline psychological distress (K10: 32/50), severe anxiety symptoms (GAD-7: 16/21), moderate depression (PHQ-9: 14/27), frequent panic attacks (3-4 per week), predominantly avoidant coping style.
- Post-Program:** Stable psychological distress scores (K10: 18/50), mild anxiety (GAD-7: 8/21), mild depression (PHQ-9: 7/27), zero panic attacks in public spaces in last 8 weeks, independent use of 4 adaptive coping strategies.
- Improvement:** ☐ None ☐ Minimal ☐ Moderate ☒ Significant

• Social Connection:

- Pre-Program:** Severe social isolation (LSNS-6: 6/18), no community participation for 18 months, avoided all social invitations, perceived low social support (MSPSS: 28/49).
- Post-Program:** Expanded social network (LSNS-6: 11/18), rejoined monthly book club (attended 3 sessions), reconnected with 2 close friends (weekly phone calls), increased perceived support (MSPSS: 40/49).
- Improvement:** ☐ None ☐ Minimal ☐ Moderate ☒ Significant

• Coping & Resilience:

- Pre-Program:** Predominantly avoidant coping styles (Brief-COPE: high behavioral disengagement, self-blame), unable to manage anxiety without avoidance, relied on sister for all social situations.
- Post-Program:** Independently uses grounding techniques (5-4-3-2-1 sensory awareness), cognitive restructuring (identifying cognitive distortions), box breathing, and self-compassion practices. Successfully completed exposure hierarchy (from "going to corner shop" to "working part-time in public-facing role").
- Improvement:** ☐ None ☐ Minimal ☐ Moderate ☒ Significant

•Functional Capacity (Employment & Community Participation):

- Pre-Program:** Unemployed for 18 months due to anxiety, unable to use public transport independently, avoided all community spaces.
- Post-Program:** Returned to part-time employment at Mackay Library (15 hours/week) since April 2026, independently uses public transport 4x/week, shops independently 2x/week.
- Improvement:** ☐ None ☐ Minimal ☐ Moderate ☒ Significant

7. RECOMMENDATIONS

THERAPEUTIC SUPPORT

Next Steps:

☒ CONTINUE PSYCHOSOCIAL THERAPY

- Program:** Phase 2 - Social Confidence & Relationship Building Program
- **Duration:** 8 sessions over 16 weeks (fortnightly)
- **Frequency:** Fortnightly (1 hour per session)
- **Focus Areas:**

- Building confidence in initiating conversations with new people
- Developing skills for maintaining friendships and setting boundaries
- Managing social anxiety in group settings (e.g., workplace social events)
- Relapse prevention and long-term maintenance strategies
- **Funding Category:** Capacity Building - Improved Daily Living (*Item 15_043_0128_1_3 - Therapeutic Supports - Counsellor*)
- **Recommended Budget:** 8 hours @ \$156.16/hour = \$1405.44 (verify current NDIS Pricing Arrangements 2025-2026)

[] **TRANSITION TO MAINTENANCE**

- **Frequency:** [X]
- **Purpose:** Consolidation of psychosocial gains and relapse prevention

[] **DISCHARGE FROM INTENSIVE THERAPY**

- Psychosocial goals achieved. Recommend maintaining a minimal contingency budget (e.g., 5 sessions/year) under Improved Daily Living for crisis support during life transitions.

Contingency/Relapse Prevention Plan:

If participant experiences a mental health setback (e.g., GAD-7 increases by >5 points, panic attacks return, or avoidance behaviors resume), recommend immediate 3-session booster block under Improved Daily Living to re-stabilize coping strategies and prevent full relapse.

PSYCHOSOCIAL SUPPORT WORKER ASSISTANCE

*(Note: Recommended support worker hours are to be funded under **Core Supports** - Assistance with Social, Economic and Community Participation, separate from this Capacity Building therapy budget).*

- **Current Hours:** 2 hours/week
- **Recommended Hours:** 4 hours/week (increase to support Phase 2 goals)
- **Focus Areas:**
 - Facilitating community connection and social skill practice: 2 hrs/week
 - Supporting attendance at book club and community events
 - Practicing conversation skills in real-world settings
 - Supporting employment sustainability: 1 hr/week
 - Pre-work preparation and anxiety management strategies
 - Post-work debrief and problem-solving workplace social challenges
 - Building independence in daily activities: 1 hr/week
 - Supported community outings (shopping, cafes) to practice skills independently
 - Gradual reduction of support as confidence increases

Expected Outcomes from Support Worker Assistance:

- Participant will independently attend book club monthly (currently requires support worker presence)
- Participant will initiate at least 1 new social connection per month
- Participant will reduce support worker hours by 25% over next 6 months as independence increases

8.SCOPE OF PRACTICE STATEMENT - CRITICAL

This report documents psychosocial therapeutic outcomes within the scope of practice of a registered counsellor (ACA Level II). It focuses strictly on mental health, emotional wellbeing, social connection, and coping capacity.

This report DOES NOT constitute:

- A Functional Behavioural Assessment (FBA)
- A Behaviour Support Plan (BSP)
- A clinical psychological diagnosis (*Participant's diagnosis of Generalized Anxiety Disorder and Major Depressive Disorder was provided by GP/psychiatrist prior to therapy commencement*)
- A neuropsychological assessment
- An assessment of physical functioning, mobility needs, or home modification requirements. (*To be assessed by an Occupational Therapist if required*).

For behavioural support needs (e.g., behaviours of concern, restrictive practices), referral to a qualified Positive Behaviour Support Practitioner is recommended.

9. CLINICIAN DECLARATION

I confirm that the information provided in this report is accurate and reflects the participant's psychosocial progress during the therapeutic program. This report was developed in collaboration with the participant, who provided informed consent for its use in NDIS planning and review processes.

- Clinician Name:** Aleksandra Gorczynska-Kolodziej
- Professional Registration:** ACA Member (Registration No: 86625)
- Qualifications:** Bachelor of Pedagogy (Special Education), Member Australian Counselling Association Level II
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Signature: *Aleksandra Gorczynska-Kolodziej*

Date: 2026-06-21

Attachments:

- Pre- and Post-Program assessment tool scores (GAD-7, PHQ-9, WEMWBS, LSNS-6, MSPSS, WHODAS, K10)
- Participant consent form for information sharing
- Service Agreement (signed)